



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نها مجدى مكرم جورجى
المحافظة : الجيزة
الحى / المدينة : سته اكتوبر
رقم البطاقة : 28602222500261 رقم التليفون : 01123083328
العنوان : ..
العمر (تاريخ الميلاد) : 22-02-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نها مجدى مكرم جورج National ID: 28602222500261	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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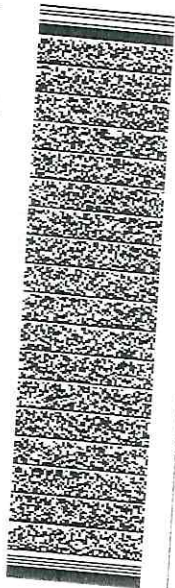
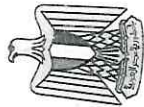
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٦/٠٧ ٧٨٦٠٢٢٢٥٠٠٣٦١
حاصلة على بكالوريوس ادارة الاعمال

أنثى
مسيحية
أفسيمة

البطاقة سارية حتى ٢٠٣٧/٠٧/٢٥



دولة فلسطين
السلطة الوطنية
للادارة المحلية



نها
مجدى مكرم جورجى
٥٠٢ دريم لاند
٦ أكتوبر ثان - الجيزة

٢٨٦٠٢٢٢٥٠٠٣٦١

LW6327621

→ ١١٢٧-٨٣٢٢٢٩
١٠-٨٣٢٢٢٩٦٩



ULTRASOUND REPORT

Name: Noha Magdy

Date : 25/08/ 2026

ميدان القديس لوقا التخصصية

Bilateral Breast Sonography revealed :

Breast composition: Prominent fibroglandular tissue element.

Findings:

► Breast parenchyma:

Left breast shows large ill defined hyoechoic mass lesion occupying all breast quadrants with multiple areas of internal vasculature and no definite internal fluid collection

No definite clusters of micro calcification.

No dilated retro-areolar lactiferous ducts.

Right breast shows no definite solid or cystic lesions

► Axilla:

Bilateral multiple enlarged axillary lymph nodes with preserved hilum and hypertrophied cortex measuring about 12 x 11 mm

Opinion:

► Large left breast mass lesion (BIRADS IVa)

Best Regards,
Dr. Manal Maher



Noha Magdy Makram George
2026-09-10
Prof.Dr. no doctor
Goustave Roussey International Egypt



nt, for biopsy.
50 %
98-100%
psy.
ion.
en malignant



malignant

98-100%

or biopsy.

50 %

Goustave Roussey International Egypt

Prof.Dr. no doctor

2026-09-10

Noha Magdy Makram George





أ.د. أحمد كمال الدري
أ.د. محمد أبو الهدى درويش
أ.د. عمر حسين عمر
أ.د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS / NOHA MAGDY MAKRAM	PATIENT ID: 1555006
DEAR SIR	STUDY DATE: 29/08/2026

BILATERAL 3D DIGITAL BREAST TOMOSYTHESIS WITH HIGH RESOLUTION ULTRASOUND, REVEALED:-

- Right breast shows normal skin, nipple, areola & subcutaneous tissue.
- Left breast shows skin thickening and subcutaneous oedema.
- Both breasts show predominant fibroglandular tissue with nodularity impressive of fibroadenosis (ACR-C).
- Left breast ill defined retro-areolar irregular mass with irregular spiculated border.
- Right axillary lymphadenopathy with preserved fatty hilum likely reactionary.

High resolution ultrasonography:

- Sonographic examination confirmed the mammographic findings and revealed left breast ill defined retro-areolar solid irregular hypoechoic mass with spiculated border measuring 5.6 X 4 cm, it shows surrounding satellites, largest at 10 O'clock 5 cm from the nipple 10 X 9 mm, with internal vascularity, impressive of highly suspicious lesion with left axillary left nodes rounded in shape with lost fatty hilum, the largest measuring about 10 X 7 mm, another left axillary ill defined hypoechoic solid irregular lesion with spiculated border measuring about 11 X 8 mm is noted.
- Bilateral scattered simple cysts ranging 2 – 5 mm.

IMPRESSION:-

- **Left breast and left axillary highly suspicious lesions, for biopsy and histopathological correlation (BIRADS 4c).**
- **Bilateral scattered simple cysts.**

MUCH OBLIGED

A. PROF. DR. MAI MOKHTAR

PROF. DR. MOHAMED ABOU EL HODA

R.A

أ.د. أحمد جمال الدري
أ.د. محمد أبو الهدي درويش
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BI-RADS 0	Need Additional Imaging Evaluation and/or Prior Mammograms For Comparison:
BI-RADS 1	Negative:
BI-RADS 2	Benign Finding:
BI-RADS 3	Probably Benign Finding – Initial Short-Interval Follow-Up Suggested: less than a 2% risk of malignancy.
BI-RADS 4	Suspicious Abnormality – Biopsy Should Be Considered: wide range of probability of malignancy (2 – 95%).
BI-RADS 4 Category 4a	low suspicion for malignancy
BI-RADS 4 Category 4b	moderate suspicion for malignancy
BI-RADS 4 Category 4c	high suspicion for malignancy
BI-RADS 5	Highly Suggestive of Malignancy. Appropriate Action Should Be Taken: with a >95% likelihood of malignancy.
BI-RADS 6	Known Biopsy Proven Malignancy. Appropriate Action Should Be Taken

ACR Classification of breast density

- a = Almost entirely fatty
- b = Scattered areas of fibro-glandular density
- c = Heterogeneously dense, which might obscure small masses.
- d = Extremely dense, decreasing the sensitivity of the mammogram.



Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي قترناتول - مصر مصر العتولفولوجي



Patient Name	نبا مجدي مكرم	Case No.	361861
Sex/Age	Female 40 Y	Patient No.	26571
Registered Date	01/9/2026 10:05:47 AM	Referred By Dr.	Interventional Radiology
Validation Date	21/9/2026 11:22:15 AM	Branch	Gustave Roussy
Reporting Date	21/9/2026 11:24:47 AM	Pathology No.	2814/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 4a).

Nature of Specimen: Tru-cut biopsy, collected on 01/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma with focal lobular component.
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): 1%.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).



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Room No. (4) - First Floor-Building No. 987- Nile Corniche- Mistr El Qadema - Cairo, Egypt

Gustave Roussy International-Egypt- Pathology Lab
جوستاف رويسى انترناتيونال - مصر معمل الباثولوجى

Patient Name	نہا مجدى مكرم	Case No.	361861
Sex/Age	Female 40 Y	Patient No.	26571
Registered Date	01/9/2026 10:05:47 AM	Referred By Dr.	Interventional Radiology
Validation Date	21/9/2026 11:22:15 AM	Branch	Gustave Roussy
Reporting Date	21/9/2026 11:24:47 AM	Pathology No.	2814/26

- Ki67%: 60%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular component, Grade 2.

IPT:

- ER: Positive (score 8/8).

- PR: Positive (score 8/8).

- Her2: Low (score 1+).

- Ki67%: 60%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

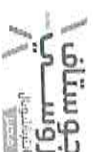
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IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:

Prof. Dr. Maha Guimei, MD, PhD.

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alvor El Qadima - Cairo, Egypt



Patient's Name: **Noha Magdy Makram George**
 Date: **2026-09-10**
 Referred by: **Prof.Dr. no doctor**
 Hospital: **Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast mass with nipple retraction

Right breast.

- Heterogenously dense breast . (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrates no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Solitary Indeterminate right axillary LN 1.3 cm x 0.9cm with eccentric hilum and cortical thickness =6 mm .

Left breast

- Heterogenously dense breast (ACR)
- non circumscribed irregular speculated dense mass occupying most of the central retroareolar region measuring about 8m x 4.3 cm with nipple retraction .
- **Ultrasound**
- the left breast show diffuse infiltrative process in the form of large irregular heterogenous hypoechoic non mass area involving the UO breast measuring about 6cm x 3.5 cm another 12 o' clock para areolar mass is seen measuring 2x1.4 cm
- the UIQ of the left breast show heterogenous hypoechoic non mas area extending in the para areolar region between 10 o'clock and 12 o'clock .
- Thickened skin reaching 10 mm with interstitial and subdermal edema and retracted nipple .
- Multiple (more than three) pathological axillary LNs are noted at levels 1, 2 & 3, one showing irregular outline ?? extra-capsular extension, largest measured 14 x9 mm with lost hilum.
- pathological supra-clavicular lymph node area seen .
- Pathological internal mammary lymph node is seen measuring 1.2 cm x1.2 cm .

Conclusion: -

- **Right breast (BIRADS 1): normal with indeterminate axillary LN .**
- **Left breast (BIRADS 5): Locally advanced malignant process with ipsilateral multilevel pathological axillary lymph nodes level 1,2 and 3 pathological supraclavicular and internal mammary LNs.**

Best Regards,
DR. Dina Sayed , M.Sc,CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %



Patient's Name:

Noha Magdy Makram George

Date:

2026-09-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

BIRADS 4: Probably malignant, for biopsy.

50 %

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

98-100%

BIRADS 6: Pathologically proven malignant



مستشفى
الجيزة

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2026-09-10
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98-100%

en malignant

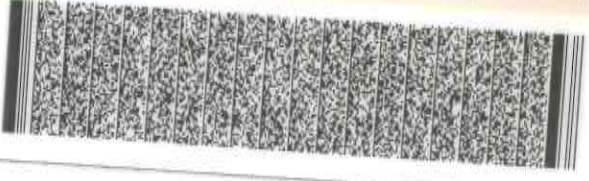
٢٨٦.٢٢٢٢٥.٠٢٦٦
٢٠٢١/٠٧
٢٠٢٣/٠٧/٢٥

البطاقة سارية حتى

أنثى مسيحية

حاصلة على بكالوريوس إدارة الأعمال

١



malignant

98-100%

50 %

or biopsy.

Goustave Roussey International Egypt

Prof.Dr. no doctor

2026-09-10

Noha Magdy Makram George



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أ.د. محمد أبو الهدي درويش
أ.د. عمر حسنين عمر
أ.د. وحيد حسين طنطاوي
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS / NOHA MAGDY MAKRAM	PATIENT ID: 1555006
DEAR SIR	STUDY DATE: 29/08/2026

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- Bilateral scattered simple cysts ranging 2 – 5 mm.

IMPRESSION:-

- **Left breast and left axillary highly suspicious lesions, for biopsy and histopathological correlation (BIRADS 4c).**
- **Bilateral scattered simple cysts.**

MUCH OBLIGED

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Reporting Date	21/9/2026 11:24:47 AM	Pathology No.	2814/26

Pathology Report

Clinical Data: Left breast mass (BIRADS 4a).

Nature of Specimen: Tru-cut biopsy, collected on 01/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:

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Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular component, Grade 2.

IPT:

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Signature:

Prof. Dr. Maha Guimei, MD, PhD.



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nisar El Qadima - Cairo, Egypt

Patient's Name:

Noha Magdy Makram George

Date:

2026-09-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: left breast mass with nipple retraction

Right breast.

- Heterogenously dense breast . (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
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Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %

Patient's Name:

Noha Magdy Makram George

Date:

2026-09-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

BIRADS 4: Probably malignant, for biopsy.

50 %

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

98-100%

BIRADS 6: Pathologically proven malignant